

HOW TO OPEN A LICENSED HOME CARE SERVICES AGENCY IN NEW YORK

A plain-language, step-by-step guide to starting a non-medical home care agency in New York — the LHCSA license, the public-need hurdle that trips up most newcomers, and how to actually get to your first client.

What this is. New York licenses non-medical home care as a Licensed Home Care Services Agency (LHCSA) under 10 NYCRR Part 766 and Public Health Law Article 36, administered by the New York State Department of Health (NYSDOH), Bureau of Home Care Licensure and Certification. New York is different from most states in one big way: it is a Certificate of Need state, so getting licensed is not just paperwork — you must show public need. This guide walks the whole path and is honest about that hurdle.

How to use it. Read Parts 1 through 4 before you spend real money — they explain the license, the public-need review that determines whether a new license is even possible in your county, and the change-of-ownership route many newcomers take instead. Parts 5 and 6 are about revenue and paying your aides — including Managed Long Term Care contracting, the Provider Choice EVV model, and New York's nation-leading wage and Wage Parity rules. Parts 7 through 9 keep you compliant. The worked examples show how it comes together in New York City and Buffalo. Verify every fee, figure, rule, and phone number before you rely on it — New York home care law changes often.

The Path at a Glance

Opening an LHCSA in New York follows a clear sequence — but Step 3 is the one that decides whether a brand-new license is realistic in your county, so read it before you commit.

1. **Confirm the license you need.** Non-medical home care = LHCSA (Part 766). Skilled, Medicare-certified home health is a separate CHHA certification (Part 763) — far harder to get.
2. **Form the business.** Register with the NY Department of State, get an EIN, open a business bank account, and secure a business office.
3. **Check public need for your county.** New York presumes NO need for a new LHCSA where five or more already serve a county. Check the DOH County Presumption of “No Need” report before you plan a new license.
4. **Choose your path.** Either apply for a new license through NYSE-CON (and rebut the presumption if it applies), or buy an existing LHCSA that serves 25+ patients — which skips the public-need review.
5. **Prepare the CON application.** Public need, financial feasibility, and the character and competence of your operators, plus your policies (this manual) — reviewed by the Public Health and Health Planning Council.
6. **Stand up screening, training & nursing.** Criminal History Record Check, exclusion screening, health/TB clearance, aide training and Home Care Registry, and registered-nurse supervision.
7. **Pass the pre-opening survey.** NYSDOH reviews your operation and conducts an on-site survey before you serve clients.
8. **Register and get paid.** Complete the annual LHCSA registration, set up EVV, and contract with MLTC plans or a CHHA to reach Medicaid clients (or start private-pay).
9. **Open and stay compliant.** Serve clients within scope with nursing supervision, keep records, report as required, and keep your license and registration current.

Part 1 — What a Licensed Home Care Services Agency Is

A Licensed Home Care Services Agency (LHCSA) is a business that provides home care services — for compensation — in a person's home. It is the license for non-medical home care in New York. An LHCSA provides personal care, home health aide services, homemaker and housekeeper services, and companion services, together with the registered-nurse supervision those services require.

What you can do

- Personal care: assistance with activities of daily living — bathing, dressing, grooming, toileting, transferring, walking, and feeding
- Home health aide services: personal care plus certain health-related tasks an HHA is trained and competency-evaluated to perform under nursing supervision
- Homemaker, housekeeper, and chore services; and companion services
- Registered-nurse assessment and supervision of the plan of care and the aides

What you cannot do (without a different license)

An LHCSA is not a Certified Home Health Agency (CHHA). A CHHA provides skilled, intermittent, Medicare-certified home health — skilled nursing, therapy — under Part 763, requires a full Certificate of Need, and is far harder and more expensive to establish. If your plan is skilled home health, that is a different and much bigger undertaking. Most people opening home care in New York want the LHCSA.

The one distinction that matters

LHCSA (Part 766) = non-medical home care: aides and nursing supervision. CHHA (Part 763) = skilled, Medicare-certified home health. They are different licenses with different processes. This guide is about the **LHCSA**.

Part 2 — The Public-Need Hurdle (Read This First)

Here is the single most important thing to understand about opening a home care agency in New York, and the thing that surprises most newcomers: you cannot simply apply and get a license. New York is a Certificate of Need (CON) state for home care. A new LHCSA license goes through the NYSE-CON system and is reviewed by the Public Health and Health Planning Council against three tests: public need, financial feasibility, and the character and competence of the operators.

The “presumption of no need”

New York applies a public-need methodology (10 NYCRR 765-1.16). Where five or more LHCSAs already actively serve patients in a county, the state presumes there is no need for another one. NYSDOH publishes a County Presumption of “No Need” report showing which counties are open and which are presumed full. In a presumed-no-need county, a new applicant must rebut the presumption with real, data-driven evidence — an underserved population, a language or cultural-competence gap, a specialized service not otherwise available. That is a high bar, and many new-license applications in dense counties do not clear it.

The change-of-ownership route

Because a brand-new license can be hard to win, many people entering New York home care take a different path: they buy an existing LHCSA. A change of ownership of an LHCSA that actively serves 25 or more patients (in the same county) is exempt from the public-need review — you are stepping into an existing, needed license rather than asking for a new one. This route has its own approval process and costs (you are buying a business), but it sidesteps the presumption of no need, which is why it is common. Have an attorney and accountant review any acquisition.

Is there a moratorium?

The 2018–2020 statutory moratorium on new LHCSA licenses has ended, and NYSDOH is accepting applications through NYSE-CON subject to the public-need methodology above. Be aware that new moratorium legislation has been proposed for a future year — verify the current status with NYSDOH before you plan a new license, because this can change.

Part 3 — The Application (NYSE-CON) and What DOH Reviews

Whether you apply for a new license or acquire an existing one, you submit through the New York State Electronic Certificate of Need (NYSE-CON) system, and NYSDOH and the Public Health and Health Planning Council review your application.

What the application must show

- **Public need** — for a new license, that the county needs another LHCSA (rebutting the presumption of no need where it applies); not required for a qualifying change of ownership.
- **Financial feasibility** — a realistic budget and working capital (generally at least a couple of months of operating expenses) to run the agency.
- **Character and competence** — background and integrity review of the owners, operators, and controlling persons, including any prior health-care operations.
- **Your policies and procedures** — a policy and procedure manual built to Part 766 (this product is that manual), reviewed by the regional office.
- **Administrative and nursing structure** — a qualified administrator and a registered nurse responsible for supervision.
- **Application fee** — a non-refundable fee (recent guidance cites about \$2,000; verify the current amount with NYSDOH).

What happens next

NYSDOH reviews the application; the Public Health and Health Planning Council acts on it; the regional office reviews your policies; and before you serve clients, NYSDOH conducts a pre-opening on-site survey of your operation. Timelines vary and are often measured in many months — plan for six to twelve months or more, and do not sign leases or hire aides as though approval is guaranteed.

Do not operate before you are licensed

Providing home care for compensation without an LHCSA license is unlawful. Wait for your license — and your registration — before serving a single client.

Part 4 — Setting Up the Business and Registering

- **Form the entity** with the NY Department of State (most agencies form an LLC or corporation) and keep corporate standing current.
- **Get an EIN** from the IRS and open a separate business bank account.
- **Secure a business office** where you administer services and keep records available for survey.
- **Buy insurance** — professional and general liability, and — as New York requires for employers — workers' compensation and disability benefits coverage.
- **Complete the annual LHCSA registration** under Public Health Law § 3605-b — separate from your license. Without current registration, you cannot operate or be reimbursed. File it with the Statistical Report through the Universal Data Collection System every year.
- **Set up payroll and EVV** to employ aides properly and capture electronic visit verification for Medicaid work.

Two separate requirements

Licensure and registration are not the same thing. You need the LHCSA license to exist, and the annual § 3605-b registration to operate and get paid. Miss the registration and you can be shut out until you cure it, with a monthly penalty.

Part 5 — Finding Clients and Getting Paid

An LHCSA reaches clients and revenue through a few channels. Know each before you build your budget, and verify current rates and contracts with the payer.

Private pay and insurance

Many LHCSA clients pay privately, by the hour, and some use long-term-care insurance or veterans' benefits. Private pay is the simplest to start with and does not require Medicaid enrollment or EVV.

Medicaid — through Managed Long Term Care or a CHHA

Most Medicaid home care in New York flows through Managed Long Term Care (MLTC) plans, which arrange community long-term care for members. An LHCSA typically serves Medicaid clients under contract — as a network provider for MLTC plans, or under contract with a Certified Home Health Agency — rather than billing Medicaid directly. This matters: getting your license does not get you clients. You must credential and contract with MLTC plans, and many plan networks are limited or effectively closed. Line up your payer relationships as deliberately as your license.

Electronic Visit Verification (EVV) — Provider Choice

For Medicaid-funded services, New York requires Electronic Visit Verification. New York uses a Provider Choice Model: you pick your own compliant EVV system — telephone, mobile app, or fixed device — as long as it meets federal and state requirements and transmits the required data to the state eMedNY EVV data aggregator. NYSDOH does not mandate a single vendor. Complete the annual EVV attestation and make sure your EVV data matches what you bill.

Paying your aides — New York's wage rules are the highest in the country

New York sets a home care aide minimum wage above the general minimum wage, and it is higher in New York City, Long Island (Nassau and Suffolk), and Westchester than in the rest of the state. On top of that, the Wage Parity Law (PHL § 3614-c) requires a supplemental wage-or-benefit amount for Medicaid-reimbursed home care aides in those downstate regions. Add overtime, spread-of-hours pay, the special live-in rules, and mandatory workers' compensation and disability coverage, and labor is your single biggest cost and compliance risk. Verify the current wage, Wage Parity, and wage-and-hour figures before you price a single hour of care.

Part 6 — Staffing, Screening and Training

- **Criminal History Record Check (CHRC)** — New York’s fingerprint-based background check for home care aides through NYSDOH, before unsupervised care.
- **Exclusion screening** — check every hire and vendor against the federal OIG and New York Medicaid exclusion lists before hire and monthly.
- **Aide training and the Home Care Registry** — home health aides must complete a NYSDOH-approved training program, pass a competency evaluation, and be listed and verified on the New York Home Care Registry; personal care aides are trained under NYSDOH-approved standards. DOH-approved HHA programs cannot charge tuition.
- **Health assessment and TB screening** — before an aide serves a client.
- **Registered-nurse supervision** — an RN assesses clients, builds and reviews the plan of care, and supervises aides with visits at required intervals. This is what makes an LHCSA more than a companion service.
- **In-service education** — at least the equivalent of six hours every six months for each aide.

Part 7 — Staying Compliant

- **Operate within scope and license** — non-medical home care only, in the counties and services on your license, with nursing supervision.
- **Keep records** — assessments, plans of care, visit and supervisory documentation, and personnel files — current, confidential, and available for NYSDOH survey.
- **Register and report every year** — the annual § 3605-b registration and Statistical Report; report changes of name, address, ownership, or services for approval.
- **Report abuse** — suspected abuse, neglect, or exploitation of a community client to Adult Protective Services (statewide 1-844-697-3505 or local DSS); 911 if in immediate danger; the Justice Center VPCR (1-855-373-2122) where a covered setting applies.
- **Meet wage, EVV, and Medicaid rules** — Wage Parity, wage-and-hour, EVV to eMedNY, and accurate billing; keep a compliance program where your Medicaid involvement requires one (OMIG).
- **Respond to surveys** — cooperate and submit a timely plan of correction for any deficiency; prevent repeat violations.

Part 8 — Your First 90 Days

1. Confirm your path — new NYSE-CON license or acquisition of an existing LHCSA — after checking the county public-need report.
2. Finalize your entity, business office, insurance (including workers' comp and disability), and bank account.
3. Adopt your Part 766 policy manual and forms and prepare your CON application package.
4. Stand up screening: CHRC, exclusion screening, health/TB clearance, and your registered-nurse supervision.
5. Line up aide training and Home Care Registry verification, and your EVV system.
6. Submit through NYSE-CON and respond promptly to DOH; prepare for the pre-opening survey.
7. Complete the annual LHCSA registration and begin credentialing with MLTC plans (or plan private-pay).
8. Once licensed, registered, and surveyed, take your first clients — and deliver reliably with nursing oversight.

Part 9 — Common Mistakes to Avoid

- Assuming you can just apply and get a license — without checking county public need first.
- Planning a new license in a presumed-no-need county without a real, data-driven case to rebut the presumption.
- Confusing an LHCSA (Part 766) with a CHHA (Part 763) — they are different licenses.
- Treating licensure and the annual § 3605-b registration as one thing — you need both.
- Winning a license but never lining up MLTC contracts — a license is not a client pipeline.
- Underestimating labor cost — New York's home care wage and Wage Parity rules are the highest in the country.
- Letting an aide start before the CHRC, health/TB clearance, training, and Home Care Registry verification are complete.
- Skipping EVV setup or letting EVV data drift from what you bill.
- Operating before the license and registration are in hand, or outside the counties on the license.

Worked Example 1 — New York City

Imagine you are opening in New York City — the largest home care market in the country, and one where the presumption of no need often applies. Here is how the pieces connect. Verify every number before you rely on it.

- **Public need:** NYC counties are dense with LHCSAs, so a new license likely faces the presumption of no need — many entrants here acquire an existing agency instead. Check the DOH county report first.
- **Referral partners:** You introduce yourself to discharge planning at hospitals such as The Mount Sinai Hospital (212-241-6500), NYC Health + Hospitals/Bellevue (212-562-4141), and NYU Langone Health (646-929-7800).
- **Aging network:** You connect with the NYC Department for the Aging through Aging Connect (212-244-6469) for community referrals and older-adult services.
- **Getting paid:** Wage Parity and the downstate home care minimum wage apply here, so price carefully; you credential with MLTC plans and set up EVV to eMedNY before taking Medicaid clients.

Worked Example 2 — Buffalo (Erie County)

Now imagine opening in Buffalo. Upstate counties can differ from downstate on public need and on wages. Verify every number before you rely on it.

- **Public need:** Check the DOH county report for Erie County — upstate counties are sometimes more open than dense downstate ones, but the presumption still governs; confirm before planning a new license.
- **Referral partners:** You build relationships with discharge planning at Buffalo General Medical Center (716-859-5600), ECMC Health Campus (716-898-3000), and Mercy Hospital of Buffalo (716-826-7000), and with rehab facilities such as Buffalo Center for Rehabilitation and Nursing (716-883-6782).
- **Aging network:** You connect with Erie County Senior Services (716-858-8526) and community hubs like the Schiller Park Senior Center (716-895-2727).
- **Getting paid:** The rest-of-state home care minimum wage applies here (lower than downstate), Wage Parity generally does not; you still credential with MLTC plans and set up EVV.

Who to Call & What to Ask

Verify each contact before you rely on it — numbers and offices change. The “Ask” note tells you what to raise with each one so your call actually moves you forward.

For...	Who to contact
License an LHCSA	NYSDOH, Bureau of Home Care Licensure and Certification · apply through NYSE-CON · health.ny.gov. Ask: the current LHCSA application and fee, whether your county has public need or requires you to rebut the presumption, and the pre-opening survey checklist.
The regulation and statute	10 NYCRR Part 766; Public Health Law Article 36 (§§ 3605, 3605-b, 361-a); public need 10 NYCRR 765-1.16. Ask: for the current text of Part 766 and the most recent Dear Administrator Letters affecting LHCSAs.
Public need / county report	NYSDOH County Presumption of “No Need” report · health.ny.gov/facilities/cons/. Ask: whether your target county is open or presumed “no need,” and — if presumed — what evidence rebuts it, or whether a change-of-ownership acquisition is the better route.
Annual registration	LHCSA Statistical Report & registration (PHL § 3605-b) via the Universal Data Collection System (UDCS). Ask: the registration deadline, what the Statistical Report requires, and the penalty for late or missed registration.
Medicaid / MLTC & EVV	NYSDOH & eMedNY — Provider Choice EVV to the state data aggregator; MLTC plan credentialing. Ask: how to credential with the Managed Long Term Care plans in your area, which plan networks are open, and how to connect your chosen EVV system to eMedNY.
Aide training & registry	NYSDOH-approved HHA/PCA training programs; New York Home Care Registry (HCR). Ask: how to verify an aide on the Home Care Registry and which approved programs train and competency-evaluate HHAs and PCAs near you.
Criminal background check	NYSDOH Criminal History Record Check (CHRC) — fingerprint-based (PHL § 3614-a / § 845-b). Ask: how to enroll as an LHCSA, submit fingerprints, and read a determination, and the supervision rules while a check is pending.
Report abuse of an adult	Adult Protective Services — statewide 1-844-697-3505 or local DSS; 911 if immediate danger; Justice Center VPCR 1-855-373-2122 (covered settings). Ask: which channel applies to your clients and how to file a report.
Complaints about a home care agency	NYSDOH Home Health Hotline — 1-800-628-5972. Ask: how a client or family files a complaint and how the agency responds to and resolves one.
Wage Parity & wage/hour	NYSDOH (Wage Parity, PHL § 3614-c) and NYS Department of Labor (minimum wage, overtime, spread-of-hours). Ask: the current home care minimum wage and Wage Parity supplement for your region, and the live-in and spread-of-hours rules.
Business registration	NY Department of State, Division of Corporations (dos.ny.gov); IRS for an EIN. Ask: how to form your LLC or corporation, appoint a registered agent, and obtain an EIN.
Veteran in-home benefits	U.S. Dept of Veterans Affairs — 1-877-222-8387 · va.gov. Ask: about Aid and Attendance and Veteran-Directed Care as private-pay funding for eligible clients.

Glossary

- **LHCSA:** Licensed Home Care Services Agency — the New York license for non-medical home care, under 10 NYCRR Part 766.
- **CHHA:** Certified Home Health Agency — the separate, skilled, Medicare-certified home health license under Part 763.
- **NYSE-CON:** the New York State Electronic Certificate of Need system through which LHCSA applications are submitted and reviewed.
- **Public need / presumption of no need:** the standard (10 NYCRR 765-1.16) presuming no need for a new LHCSA where five or more already serve a county.
- **PHHPC:** the Public Health and Health Planning Council, which reviews and acts on home care applications.
- **MLTC:** Managed Long Term Care — the Medicaid managed-care model through which most community long-term home care flows.
- **Wage Parity:** PHL § 3614-c — required supplemental wage/benefit levels for Medicaid-reimbursed home care aides downstate.
- **EVV / eMedNY:** Electronic Visit Verification; New York's Provider Choice model transmits EVV data to the state eMedNY aggregator.
- **Home Care Registry (HCR):** the New York registry on which qualified aides are listed and verified.

Core Rules & References

- 10 NYCRR Part 766 — Licensed Home Care Services Agencies (operational standards)
- 10 NYCRR 765-1.16 — public need methodology (presumption of no need)
- Public Health Law Article 36 — § 3605 (licensure), § 3605-b (annual registration), § 361-a (application fee)
- 10 NYCRR Part 763 — Certified Home Health Agencies (the separate skilled license, for contrast)
- PHL § 3614-a; § 845-b — Criminal History Record Check for home care aides
- PHL § 3614-c — Home Care Worker Wage Parity
- 21st Century Cures Act § 12006 — federal Electronic Visit Verification mandate (NY Provider Choice model)
- NY Labor Law & NYS DOL wage orders — home care minimum wage, overtime, spread-of-hours, live-in rules

This guide is general educational information, not legal, tax, or accreditation advice. New York rules, fees, programs, wage figures, and phone numbers change — verify each with NYSDOH, the NYS Department of Labor, and the relevant payer or authority before you rely on it.